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FOCUS GASTROENTEROLOGY

16 April 2026

Dr Adrian Dragovic
Medical One Moonee Ponds

Dear Dr Dragovic,

Re: Elizabeth Bennett
DOB: 1989-05-06
Address: 7/201 Abbotsford Street, NORTH MELBOURNE, VIC, 3051

Summary

- **Resolution of gastro-oesophageal reflux and pharyngeal symptoms on amitriptyline.**
- **Management of treatment-emergent constipation and sweet cravings.**
- **Plan for staged dose reduction or transition to nortriptyline should side effects persist.**
- **Introduction of regular macrogol (Osmolax) for bowel management.**
- **No action required by you.**

I had the pleasure of conducting a review with Elizabeth Bennett to follow up on her progress. Thank you for your updated referral letter. You will be pleased to know that her previous symptoms of heartburn have effectively disappeared since commencing her current medication regimen. Furthermore, she reports that her throat irritation and post-nasal drip have completely resolved, and her symptoms of bloating are now significantly less frequent.

Elizabeth is currently taking 10 mg of amitriptyline daily, which has been highly effective in modulating her visceral sensitivity and drying the secretions that were likely contributing to her pharyngeal symptoms. However, she is experiencing notable anticholinergic side effects. Specifically, she reports significant constipation, passing hard faeces. This has occasionally required the reactive use of enemas and has negatively impacted her mood. She also noted a distressing increase in sweet cravings and associated weight gain.

Impression and Plan To manage the constipation, I have advised Elizabeth to transition from reactive treatment to a regular bowel programme. Given her aversion to the taste of Movicol, I have

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320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

recommended Osmolax, which is a macrogol that allows for dose flexibility to ensure stools remain soft and easy to pass. We also briefly discussed magnesium oxide (S. Pellegrino Magnesia) as a secondary option.Regarding the amitriptyline, we have formulated a staged plan to address the side effects. Elizabeth will remain on 10 mg for another month to observe if the cravings stabilise. If they do not, she will reduce the dose to 5 mg. Should the constipation or weight gain remain intolerable, she has been provided with a prescription to switch to nortriptyline 10 mg at night. I have cautioned her that while nortriptyline is generally less constipating and less sedating, some patients experience a recurrence of reflux symptoms on this medication. We have scheduled a telehealth review in six months to evaluate her progress and the efficacy of these adjustments.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: A/Prof Michael Hii